



OFFICIAL

Electroconvulsive Therapy Procedure

1. Scope

Site	Operational Area	Applicable to
Armada Health Service;	Mental Health Services Anaesthetics	Medical and Nursing

2. Purpose

This procedure prescribes the details for the facilitation of the safe and effective delivery of Electroconvulsive Therapy (ECT) for Armada Health Service (AHS) consumers in accordance with the legislative requirements and the standards set out by the Chief Psychiatrist.

The [Electroconvulsive Therapy Policy](#) is required to give effect to this procedure.

3. Procedure

3.1 Referral

All referrals for ECT are to be received by the ECT Nurse Coordinator. It is the responsibility of the ECT Prescriber and treating team to complete the Pre ECT-Evaluation. Once the Pre ECT-Evaluation has taken place the ECT Nurse Coordinator will book the consumer into theatre for the prescribed course of treatment. (Refer to [Appendix 1](#) and [Appendix 2](#): ECT Treatment Flow Charts). Referrals received from outside AHS will be reviewed on a case-by-case basis and must be approved by the Psychiatry Head of Service.

3.1.1 Pre ECT evaluation (to be completed by the ECT Prescriber)

A pre-ECT evaluation and workup should be carried out within 7 days prior to the commencement of ECT for both inpatients and day patients. This will include the following:

- A thorough psychiatric evaluation, history and physical examination.
- A further psychiatric opinion if requested by the consumer.
- Previous ECT history and supporting documentation.
- A medication review.
- An evaluation of dentition for the presence of dentures and dental problems that could affect the use of the bite block.
- An electrocardiogram is essential.
- Serum electrolytes – Serum potassium and sodium should be corrected prior to commencing ECT if possible.
- Other routine lab investigations are not mandatory and should be guided by the consumer's history and a physical examination.
- Chest X-ray is not routinely required unless indicated.

Before referencing this procedure document please ensure that you have the latest version from the [Policy Library](#) information hub page.

- Neuroimaging should be considered in patients with abnormal neurological examinations/clinical indication/recent stroke/raised intracranial pressure/intracranial space occupying lesion.
- In cases where doubt exists about a consumer's suitability for ECT, such as recent myocardial infarction or major cardiac event, the opinion of a specialist physician will be sought.
- All consumers must have a documented treatment, support and discharge plan (TSD) in the health record. Where possible the consumer will be involved in this process and sign the plan.
- A record of the consumers wishes, to the extent that it is practicable to ascertain them, must be documented in the health record including any:
 - contemporaneous wishes expressed by the consumer
 - relevant treatment decision in an Advance Health Directive (AHD)
 - relevant term of an enduring power of guardianship
 - other things that the medical practitioner considers to be relevant to ascertaining the consumer's wishes.

3.1.2 Pre-anaesthetic consultation

- A Pre- anaesthetic assessment is mandatory.
- Anaesthetic assessment must be completed prior to the administration of ECT by an anaesthetist. It should be reviewed prior to each treatment and updated as required as per the [Administration of Anaesthesia for Electroconvulsive Therapy Guideline](#).

3.2 Consent

Written and valid consent is required for ECT administration. Due consideration will be given to any Advanced Healthcare Directives regarding ECT Treatment.

The prescribing psychiatrist or medical practitioner must ensure that the appropriate consent and approvals comply with the Mental Health Act 2014.

Consent begins when the treatment is first recommended and does not end until the treatment is completed. Consent must be freely and voluntarily given. A failure to offer resistance to treatment does not of itself constitute consent to treatment.

The consent should be obtained or renewed every 3 months or every 12 treatments, whichever comes first as per the [EMHS Consent Policy](#).

New consent must be obtained if there are substantial changes to treatment (eg, electrode placement or pulse width)

3.2.1 Capacity to give informed consent

As per the [WA Health Consent to Treatment Policy](#), the consent process should be conducted face-to-face where possible and should be considered as a series of steps:

Step 1: Determine which health professional is responsible for seeking consent.

Step 2: Assess the consumer's capacity.

Step 3: Provide sufficient information so the consumer can make an informed decision.

Step 4: Verify that the consumer understands the information given and all their queries have been addressed.

Step 5: Seek a decision from the consumer about the proposed treatment.

Step 6: Document consent.

A consumer can give informed consent if they have capacity to do so. The consumers support persons can be included in the consent process.

The capacity assessment includes assessment of the following:

- Understand the explanation and information that has been provided about the treatment, any alternative treatments, and any risks involved.
- Understand the matters involved in making a treatment decision.
- Understand the effect of the treatment decision.
- Weigh up the different factors in the treatment decision.
- Communicate the decision in some way.

The needs of consumers, their families and carers including cultural and linguistic requirements should be considered in providing ECT treatment that is accessible and responsive to their specific needs.

With regard to Aboriginal and Torres Strait Islander consumers, consultation should extend to Aboriginal and Torres Strait Islander (ATSI) workers, traditional healers, elders, or other significant persons from the consumer's community.

3.2.2 Carer involvement

Prior to the commencement of ECT, consumers, and their personal support person/s as appropriate, will be provided with information related to ECT. The following will be given/ made available:

- AKG brochure: Electroconvulsive Treatment (ECT): Information for consumers, family, and carers.
- Mental Health Commission brochure: [Receiving Treatment for a Mental Illness](#) – available in other languages and alternative formats.
- Mental Health Commission handbook: [Family and Carer Handbook to the Mental Health Act 2014](#).
- A trained interpreter for non-English speaking consumers/carers to enable them to enquire and discuss all aspects of ECT in their native language.

3.2.3 Voluntary consumers

- ECT can be administered to an adult voluntary consumer with the informed consent of the consumer, or a person authorised by law to provide consent such as a guardian as per the WA Mental Health Act 2014 (s. 197).

Consent must be verbally confirmed prior to every treatment. A voluntary consumer has the right to withdraw their consent to ECT at any time and this should be discussed with the patient and documented in the medical record.

3.2.4 Mental health tribunal approval

- ECT can be administered to an adult involuntary consumer or Mentally Impaired Accused (MIA) with approval from the Mental Health Tribunal.

- The ECT Prescriber is required to submit the application to the Mental Health Tribunal in writing. Treatment cannot commence until approval has been obtained.
- If the mental health tribunal approves a course of ECT the consent for administration of anaesthetic is included as part of the ECT.

3.2.5 Emergency ECT

- Emergency ECT can be provided to an adult involuntary consumer or Mentally Impaired Accused with approval from the Chief Psychiatrist or delegate as per the [Chief Psychiatrist Schedule of Delegations: Approval for emergency electroconvulsive therapy](#) in accordance with the WA Mental Health Act 2014 (s. 199).
- A Psychiatrist seeking the Chief Psychiatrist or delegate's consent to provide emergency ECT must via a telephone call or email provide the following details:
 - The involuntary consumer's demographic details
 - Confirmation that the consumer is an adult
 - The clinical reasons for seeking to treat the consumer with Emergency ECT, which demonstrates that; It is required to save the consumer's life; or
 - Because there is an imminent risk of the consumer behaving in a way that is likely to result in serious physical injury to themselves or another person.
 - On receipt of a request the Officer exercising the Chief Psychiatrist's power of delegation must respond in writing via email and copy the email in to: monitoring@ocp.wa.gov.au
- Note that the Chief Psychiatrist or delegate is only authorising one, or in exceptional circumstances, two treatments until the Mental Health Tribunal can be convened.
- The approval must be provided before any treatment is given
- The ECT must be performed at an approved mental health service.

3.2.6 ECT regarding children

- Emergency ECT cannot be provided to a child.
- ECT is prohibited for a child less than 14 years of age.
- ECT must be approved by the Mental Health Tribunal for all children who have reached 14 years of age but are under 18 years of age (mature minor).
- A voluntary child over the age of 14 years can consent if he or she has capacity, but approval is still required by the Mental Health Tribunal.
- For the voluntary child who does not have capacity, consent can be given by a person authorised by law (e.g. parent/guardian) but approval is still required by the Mental Health Tribunal.
- A child who is an involuntary consumer or mentally impaired accused over the age of 14 years can be given ECT with the approval of the Mental Health Tribunal.

3.3 Treatment

ECT can only be performed if Armadale Mental Health Service has met all the standards and achieved the approval of the Chief Psychiatrist as required under the WA Mental Health Act 2014 (s. 544).

The number of treatments for ECT is determined on an individual basis. A full course of ECT is 6 - 12 treatments and may result in hospital admission of at least one month. The consumer's

treating team is responsible for the overall care of the consumer. The ECT treating team is only responsible for providing the ECT treatment.

- The ECT Coordinator or a delegate must be present at each treatment session.

3.3.1 Setting

- ECT will be administered in the operating theatre suite at Armadale Health Service on Monday, Wednesday and Friday commencing at 0800 (excluding public holidays that fall on these days).
- Stage I recovery will take place in the post anaesthetic care unit (PACU), after which the consumer will be transferred to the relevant mental health ward or same day unit for stage II recovery.
- Adequate provision of privacy for the consumer receiving ECT will be ensured, with adequate space for the anaesthetic and ECT equipment, as well as for staff to assist with the procedure and/or any medical emergencies.
- **Visitors to theatre:** Consumers with a special need or requirement shall have a carer/support person to accompany them to theatre in accordance with the [AKG Visitor Access Procedure](#).

3.3.2 Equipment

The ECT machine (Thymatron IV™) will:

- Enable electroencephalogram (EEG) monitoring of the seizure.
- Be registered with the Therapeutic Goods Administration.
- Be safety checked and maintained by the Health Technology Management Unit as per EMHS [Medical Equipment- General Information](#)
- Be reviewed annually against current research evidence and updated when no longer able to meet best practice requirements.
- Anaesthetic equipment will be registered and within the standards required by ANZCA.
- Emergency equipment is to be maintained, checked, and situated close to the treatment and recovery area.

3.3.3 Patient preparation

Clinical handover will be provided at every change of care from one clinician to another using ISOBAR format as per the [Communicating for Safety Procedure](#).

Nursing staff on the Mental Health Unit shall attend to/follow up the following:

At least 24 hours prior to the scheduled ECT treatment:

- The ECT work up and prescription medical record (PAKMR100.1) is to be checked that they are complete.
- The requirement for security to attend or standby will be identified through a risk assessment and documented in the health record.
- Ensure the consumer's Treatment, Support and Discharge plan (TSD) is clearly documented in the health record.

On the evening shift prior to the scheduled ECT treatment:

- Jewellery is to be removed and stored as per AMHS Clinical Guidelines [Patient Property in Mental health Wards](#). This is to be offered back to consumer after ECT treatment. Any body piercings will be taped or removed prior to ECT.
- Personal hygiene will be discussed with the consumer the evening prior to ECT. Ensure the consumer:
 - Has washed their hair
 - Is reminded not to wear makeup, creams, or talcum powder.
 - Does not have varnish on their nails.
 - Document a discussion with the consumer regarding the fasting procedures.

On the night shift prior to the scheduled ECT treatment:

- If the consumer is a smoker, they will be advised of the requirement for cessation of smoking from midnight (24:00) and for two hours post ECT treatment. Nicotine Replacement Therapy (NRT) is to be offered to the consumer.
- Hypertensive medications are to be dispensed in the morning at least an hour prior to ECT or as per physician's order.
- Record the consumer's vital observations including **blood sugar levels** if the consumer is diabetic in the PAKMR100.3 Nursing Record.
- Abnormalities in vital observations and any other overt/identified medical problems are to be communicated to the Treating Team and ECT Nurse Coordinator.

Nursing staff on the Mental Health Unit and in the Operating Theatre Suite shall attend to and follow up the following:

On the day of ECT treatment:

Check that the consumer has 2 name bands (wrist and opposite ankle), as per [Identification of Patients in the Armadale Mental Health Service Procedure](#). The name bands should be verified by a second nurse and identified along with the health record, as per EMHS Patient Identification Policy. The consumer's allergies should be clearly documented as per the EMHS Patient Identification and Procedure Matching.

- Allow the consumer to retain their contact lenses/glasses, hearing aids, dentures and prostheses until immediately before the ECT procedure.
- Advise the consumer to void his or her bladder and bowels prior to ECT.
- Advise the consumer to wear a hospital gown and maintain undergarments.
- A Nurse should transfer the consumer to theatre in a wheelchair with accompanying security if required.
- Provide the health record and correct paperwork to the Theatre staff.
- Theatre staff will undertake theatre processes for accepting consumers and check that all paperwork is complete before commencement of ECT.

3.3.4 Treatment

A Team time out must be conducted before the anaesthetic to check and ensure-

- correct patient
- correct consent
- correct electrode placement

- correct pulse width
- correct dose
- correct anaesthetic drugs and doses

All non-essential communication should be ceased during Team Time Out. Include any anticipated procedural or clinical difficulties in the team time out

- The team time out must be documented
- Confirm with the anaesthetist that the patient is fully anaesthetised, and that adequate muscle relaxation has been achieved and that the bite block has been inserted

Post anaesthetic Team Time Out – The anaesthetist must say “anaesthetic delivered, ECT can proceed” The ECT interventionalist must say “ECT will be given/delivered now”

Detailed instructions about electrode placement and operation of the ECT machine (Thymatron IV™) are located in the procedure manual ‘Armadale Health Service Procedure for Administration of Electroconvulsive Therapy in Theatre’, located with the ECT machine.

The ECT Interventionalist will ensure that the ECT machine is functioning by following the guidelines indicated in the procedure manual.

Anaesthesia will be administered as per the Armadale Health Service guideline [Administration of Anaesthesia for Electroconvulsive Therapy](#)

Electrode Placement

- The ECT Prescriber will determine the electrode positioning for the treatment and must specify this and number of treatments in the Consent (PAKMR100.1).
- During the course of treatment any change of electrode placement shall be discussed with the consumer/guardian/carer and documented in the PAKMR100.1.

Pulse Width

- The ECT Prescriber will determine the pulse width for the treatment and will specify this in the ECT prescription in the PAKMR100.1. Pulse width can be prescribed as 0.3 ms, 0.5 ms, or 1.0 ms (DGx setting).

Stimulus Dose

- Choice of starting dose will usually be the ‘titration method’. The following ECT Titration Guidelines will be followed:

Level	Titration Dose (%)	UL Treatment Dose (%)	BL Treatment Dose (%)
1	2	10	3
2	5	25	8
3	10	50	15
4	15	75	25

Level	Titration Dose (%)	UL Treatment Dose (%)	BL Treatment Dose (%)
5	25	125	45
6	35	175	55
7	50	200	75

Threshold Determination

- Start at:
- Female: Level 1 for UL, level 2 for BL
- Male: Level 2 for UL, level 3 for BL. Increase by one level if re-stimulation is necessary.
- If there is no adequate seizure after 3 stimulations increase TWO levels for the fourth stimulus and if successful drop one level for the first stimulus at the next treatment session to continue titration before proceeding to treatment dose.
- If no seizure after fourth stimuli, abort session and increase by one level for the first stimulus at next session to continue titration.

Successive Treatment

- After establishing the lowest level needed to produce an adequate seizure increase by:
For: High Dose UL (approximately 5 x ST)
Low Dose BT (approximately 1.5 x ST)
High Dose BT (approximately 2 – 2.5 x ST)
- Whenever further increases in stimuli are indicated increase by 50% of the current treating dose.
- If the consumer has been treated with ECT previously then starting dose of both ECT and anaesthetic will be determined by review of the database of their previous treatment.
- Apart from during stimulus titration, no more than 3 stimuli per patient should be delivered during an ECT treatment session due to the risk of side effects.

3.3.5 Monitoring during ECT treatment

The following monitoring will occur throughout the procedure.

Seizure monitoring

Routinely left and right para-sagittal sites will be used for seizure monitoring. Effectiveness of a seizure will be monitored by measuring:

- Duration of seizure
- Higher amplitude spike and wave activity during the seizure
- Post-ictal suppression
- Ictal motor activity

Cardiovascular monitoring

- Heart rate and ECG rhythm
- O2 saturation
- Blood pressure

- Inspiratory and expiratory O2 and CO2 levels

3.3.6 Workflow

Refer to [Appendix 1](#) and [Appendix 2](#) ECT Treatment Flow Charts.

3.4 Recovery

Clinical handover will be provided at every change of care from one clinician to another using ISOBAR format as per the [Communicating for Safety Procedure](#).

3.4.1 Stage I: PACU-based recovery

- Theatre nursing staff to monitor and record the consumer's vital observations for 30 minutes including sedation/conscious levels on the Post Anaesthesia Care Unit Record - PAKMR100.3 – ECT Nursing Record (Stage I Recovery).
- The consumer is to remain in the post anaesthetic recovery area for as long as deemed necessary by recovery staff but no less than 30 minutes.
- When the consumer has met recovery discharge criteria, theatre nursing staff are to contact the relevant ward and complete a nurse to nurse handover using the PAKMR100.2-ECT Anaesthetic Record and PAKMR100.3 – ECT Nursing Record (Stage I Recovery)
- The consumer is to be transferred back to the ward on discharge from the recovery suite in a wheelchair by a nurse and accompanied by another staff member (such as a PCA).
- The ECT Nurse Coordinator will remain in theatre/ recovery area until the last consumer being treated is discharged.

3.4.2 Stage II: Mental health ward / same day unit-based recovery

- The consumer's vital observations are to be monitored and recorded in the PAKMR100.3-ECT Nursing Record (Stage II Recovery) every:
 - 15 minutes for the first hour.
 - 30 minutes for the second hour.
- Observations should continue 30/60 until the consumer meets the following criteria:
- Conscious state is awake, alert and orientated.
- Vital observations are stable and within normal limits as established pre-ECT.
- They are able to tolerate diet/fluids and have passed urine.
- They are able to mobilise according to pre-ECT state.
- If relevant, assessment and management of nausea and/or post ECT aches/pain have been carried out; and
- A medical officer advises the observations can be ceased.
- The consumer should not smoke for at least two hours following ECT Treatment.
- If the consumer is granted leave, transport needs to be pre-arranged as they should not drive a vehicle or travel alone in public transport within 24 hours of receiving ECT. They should remain in the company of a responsible adult during the leave period. The consumer must have met all criteria of stage II recovery.

3.5 Post treatment care

3.5.1 Monitoring of ECT treatment

- Any issues during ECT recovery, including diet and fluid intake must be documented in the health record on return to the ward and the attending medical officer made aware as soon as practicable.
- The medical officer is to review the consumer within 2-4 hours after ECT treatment in the ward and determine if vital sign monitoring should continue.
- Further monitoring should be recorded on the AKMR140.3 Adult Observation and Response Chart.
- The medical officer should document the review in the health record.

3.5.2 Evaluation of ECT treatment

Baseline assessment of a consumer's symptoms will be documented before ECT and specific target symptoms will be identified.

- The ECT Prescriber will review the consumer (inpatient) in line with [AKG Assessment and Admission Documentation Requirements Policy](#).
- For the consumer receiving ECT treatment as a day patient, an evaluation of treatment should be conducted on a weekly or as needed basis but no less than monthly. For consumers receiving maintenance treatment the frequency of assessment can be fortnightly or monthly.
- The Treating Team should alert the ECT Interventionalist to any change in medication and consumer status since the consumer's last treatment.
- The frequency, number of treatments and ECT prescription will be reviewed after each ECT treatment.
- The ECT Interventionalist and the ECT Prescriber will set parameters for the next ECT treatment and document in the PAKMR100.1 and consumers health record.
- The consumers Treatment, Support and Discharge plan (TSD) will be updated and signed with the consumer.
- The following rating scales can be used for evaluation of consumers cognitive changes before, during (halfway through the course) and after treatment:
 - Montreal Cognitive Assessment (MOCA)
 - Orientation Questionnaire (to be completed 30 minutes post ECT)
 - Clinical Global Impression Scale
 - Brief Psychiatric Rating Scale (BPRS)
- Consumers should be offered the following self – evaluation scales
 - Patient Health Questionnaire (PHQ-9)
 - Likert scale
 - ECT patient feedback form

3.6 Additional considerations

3.6.1 Day patients

Patients identified as suitable for outpatient ECT must meet the requirements of the [Chief Psychiatrist Guidelines for the use of ECT in WA](#) 5.2 Suitability for outpatient ECT (pg 43).

The Consultant prescribing ECT (ECT Prescriber) should follow the Referral and Consent procedures as described above.

- The Treating Team in consultation with the ECT Nurse Coordinator will ensure the pre ECT requirements are carried out.
- A consumer having their first ECT can be admitted overnight onto a Mental Health Ward for adequate monitoring of ECT and anaesthetic side effects. If there are no concerns following this, the consumer may be able to continue treatment as a day patient.
- The ECT Nurse Coordinator in collaboration with the designated Care Coordinator will organise admission to Same Day Unit on the morning of the consumer's ECT treatment.
- Treatment and Recovery will be as described above.
- Discharge Criteria will be as described below.
- Special arrangements will be made with the Mental Health Inpatient Wards to accommodate ECT for day patients during the 2-week Christmas period when the Same Day Unit is closed.

3.6.2 Discharge criteria

Where the ECT consumer is approved for discharge:

- Consumers will be allowed to go home 2 - 4 hours after administration of ECT and following assessment by the ECT Interventionalist or his/her delegate.
- A full mental state and risk assessment will be completed and documented in the integrated progress notes prior to discharge. As the consumer is not to drive a vehicle or travel alone in public transport within 24 hours of receiving ECT, transport home needs to be pre-arranged.
- The discharge instructions for ECT will be provided to the consumer along with emergency contact details and follow-up arrangements.
- The consumer/support person will sign the discharge instructions or day patient's checklist to indicate they have received the following education and instructions in writing:
 - During ECT you will receive a general anaesthetic and therefore the following standard precautions apply. For the next 24 hours, you must:
 - Be in the company of a responsible adult
 - Not drive a vehicle or travel alone in public transport
 - Not sign any legal documents or make important decisions
 - Not operate machinery or appliances with safety regulations
 - Not be left in sole charge of young children
 - Not consume alcohol
- The consumer must have someone organised to stay with them overnight.

3.6.3 Continuation and maintenance ECT

The need for continuation or maintenance ECT must be made in line with the therapeutic guidelines (Chief Psychiatrist's Guidelines for the use of Electroconvulsive Therapy in Western Australia 2024 Guideline 10 Continuation and maintenance ECT pg 70-72).

A psychiatrist proposing continuation or maintenance ECT should explain the rationale for such proposed treatments to the consumer and discuss the evidence to justify its use in the consumer's particular circumstances including alternatives to the treatment, and the possible risks and benefits of proceeding with each alternative.

3.7 Documentation and reportable data

Documentation of the ECT process is carried out in accordance with the practice standards and guidelines set down by the Office of the Chief Psychiatrist, with specified medical record forms to be used.

3.7.1 Electroconvulsive therapy documentation

In addition to documentation in the consumer's health record, the following will be completed and documented in the relevant Electroconvulsive Therapy forms:

- Referral for ECT – PAKMR100
- ECT Medical Record – PAKMR100.1
- ECT Anaesthetic Record – PAKMR100.2
- ECT Nursing Record – PAKMR100.3

3.7.2 Other documents

In addition to documentation in the consumer's health record, the following will be completed and documented:

- Care Planning – Treatment, Support and Discharge Plan (TSD)
- Surgical Team Time Out Procedure – AKMR93.6 Surgical Safety Checklist
- Discharge – Discharge Information for Electroconvulsive Therapy
- Evaluation of treatment – ECT Effectiveness Monitoring Scales, ECT Patient Satisfaction Questionnaire
- Discharge Summary
- Care Transfer Summary

The ECT Nurse Coordinator will ensure that all ECT data is entered into the relevant database by the end of each shift.

3.7.3 ECT evaluation

The treating Consultant Psychiatrist should enter the following information in the consumer's health record:

- A summary of overall therapeutic outcome including adverse effects experienced as a result of the ECT course or series.
- The rationale for choice of endpoint.
- A plan for post-ECT clinical management and any plans for follow-up of adverse effects.

In addition, for each consumer, the following information should be kept:

- Age
- Sex
- Unit Medical Record Number (UMRN)
- Whether this is an index course or maintenance course of ECT.
- Whether this is an inpatient or day patient.
- Whether the consumer is voluntary or involuntary
- Dates of treatment.

- Name and designation of the referring psychiatrist.
- Name and designation of the psychiatrist, psychiatric medical officer or psychiatric registrar administering the treatment.
- Any side effects or complications that occurred during the course of ECT.
- The primary diagnosis as a reason for requiring ECT.
- The indications for ECT.
- A statement about previous ECT response.
- Elements of pre-ECT workup completed.
- Outcome measures, including a cognitive scale, the Clinical Global Impression Scale, a depression scale, and a consumer satisfaction measure (qualitative or quantitative).
- The reason ECT is ceased.
- The number of unilateral and bilateral treatments.
- Treatment location (name of hospital).

3.7.4 Reportable data

ECT Administration

- If the consumer is an MIA, the ECT Prescriber must report the provision of any ECT to the Mentally Impaired Accused Review Board, accompanied by a copy of the Mental Health Tribunal's approval or the Chief Psychiatrist's approval for emergency ECT.
- Each time an emergency ECT treatment is given the Office of the Chief Psychiatrist must be advised.

Incidents

- Clinical incidents must be reported using the clinical incident reporting system [Clinical Incident Notification Form](#) in Datix CIMS and managed in accordance with the [Clinical Incident Management Policy](#).
- Serious incidents relating to the practice of ECT will be promptly reported by the ECT Director to the Chief Psychiatrist through the Clinical Incident Management System (CIMS).

ECT Data

The services ECT Director and the ECT Nurse Coordinator maintain an ECT Register and ECT database and complete monthly statutory reporting for the Office of Chief Psychiatrist (Form 13).

4. Policy context

- The following documents are required to give effect to this policy:
- Legislation
 - [Western Australia Mental Health Act 2014](#)
 - [WA Guardianship and Administration Act \(1990\)](#)
 - [Health Practitioner Regulation National Law \(WA\) Act 2010](#)
 - [Western Australia Carers Recognition Act 2004](#)
- HDWA
 - [Clinical Incident Management Policy](#)
 - [Clinical Alert Policy](#)

- [Clinical Handover Policy](#)
- [Consent to Treatment Policy](#)
- [Complaints Management Policy.](#)
- EMHS
 - [Medical Equipment- General Information](#)
- AKG
 - [Identification of Patients in the Armadale Mental Health Service Procedure](#)
 - [Patient Property in Mental health Wards Procedure](#)
 - [Communicating for Safety Procedure.](#)
 - [Assessment and Admission Documentation Requirements Policy.](#)

5. Supporting information

The following documents support the implementation of this policy:

- [Chief Psychiatrist's Standards for the administration of Electroconvulsive Therapy \(ECT\) in WA 2025](#)
- [The Chief Psychiatrist's Guidelines for the use of Electroconvulsive Therapy in Western Australia 2024](#)
- [Electroconvulsive Therapy Policy](#)
- [Administration of Anaesthesia for Electroconvulsive Therapy Guideline](#)
- [Family and Carer Handbook to the Mental Health Act 2014.](#)
- [Mental Health Commission Receiving Treatment for a Mental Illness](#)

6. Compliance, monitoring and evaluation

The ECT Review Committee will be responsible for ensuring compliance monitoring, internal auditing, and quality improvement activities. The processes, implementation, and developments in ECT delivery will be reviewed on a regular basis. Consumers, carers, advocates, and other stake holders will have this forum to provide input to improve this area.

As part of the quality improvement process, an annual audit should include one or more of the following:

- Documented consent.
- Pre-ECT checklists.
- ECT treatment forms.
- Side effects and complications.
- Basic treatment outcomes.
- Consumer and family education activities.
- Consumer satisfaction.
- Treatment, Support and Discharge Plan.

7. Relevant standards

National Safety and Quality Health Service (NSQHS) Standards (second edition):

- Standard 2: Action 2.4: The health service organisation ensures that its informed consent processes comply with legislation and best practice
- Standard 2: Action 2.5: The health service organisation has processes to identify:
 - a. The capacity of a patient to make decisions about their own care
 - b. A substitute decision-maker if a patient does not have the capacity to make decisions for themselves
- Standard 2: Action 2.10: The health service organisation supports clinicians to communicate with patients, carers, families and consumers about health and health care so that:
 - a. Information is provided in a way that meets the needs of patients, carers, families, and consumers
 - b. Information provided is easy to understand and use
 - c. The clinical needs of patients are addressed while they are in the health service organisation
 - d. Information needs for ongoing care are provided on discharge
- Standard 6: Action 6.7: The health service organisation, in collaboration with clinicians, defines the:
 - a. Minimum information content to be communicated at clinical handover, based on best-practice guidelines
 - b. Risks relevant to the service context and the particular needs of patients, carers, and families
 - c. Clinicians who are involved in the clinical handover

National Standards for Mental Health Services (2010):

- Standard 1: Rights and Responsibilities
- Standard 3: Consumer and Carer Participation
- Standard 6: Consumers
- Standard 8: Governance, leadership, and management

[Chief Psychiatrist Standards for the Administration of Electroconvulsive Therapy \(ECT\) in Western Australia 2025](#)

- Standard 1: ECT Service delivery
- Standard 2: Consent and the legal framework
- Standard 3: Treatment provision
- Standard 4: Safety, quality and reporting

8. Policy owner

Service Director - Mental Health

Enquiries relating to this policy may be directed to:

Title: ECT Nurse Coordinator
Department: Mental Health
Email: Jane.Priest@health.wa.gov.au

9. Review

This policy will be reviewed and evaluated as required to ensure relevance and currency. At a minimum it will be reviewed within one (1) year after first issue and at least every three (3) years thereafter.

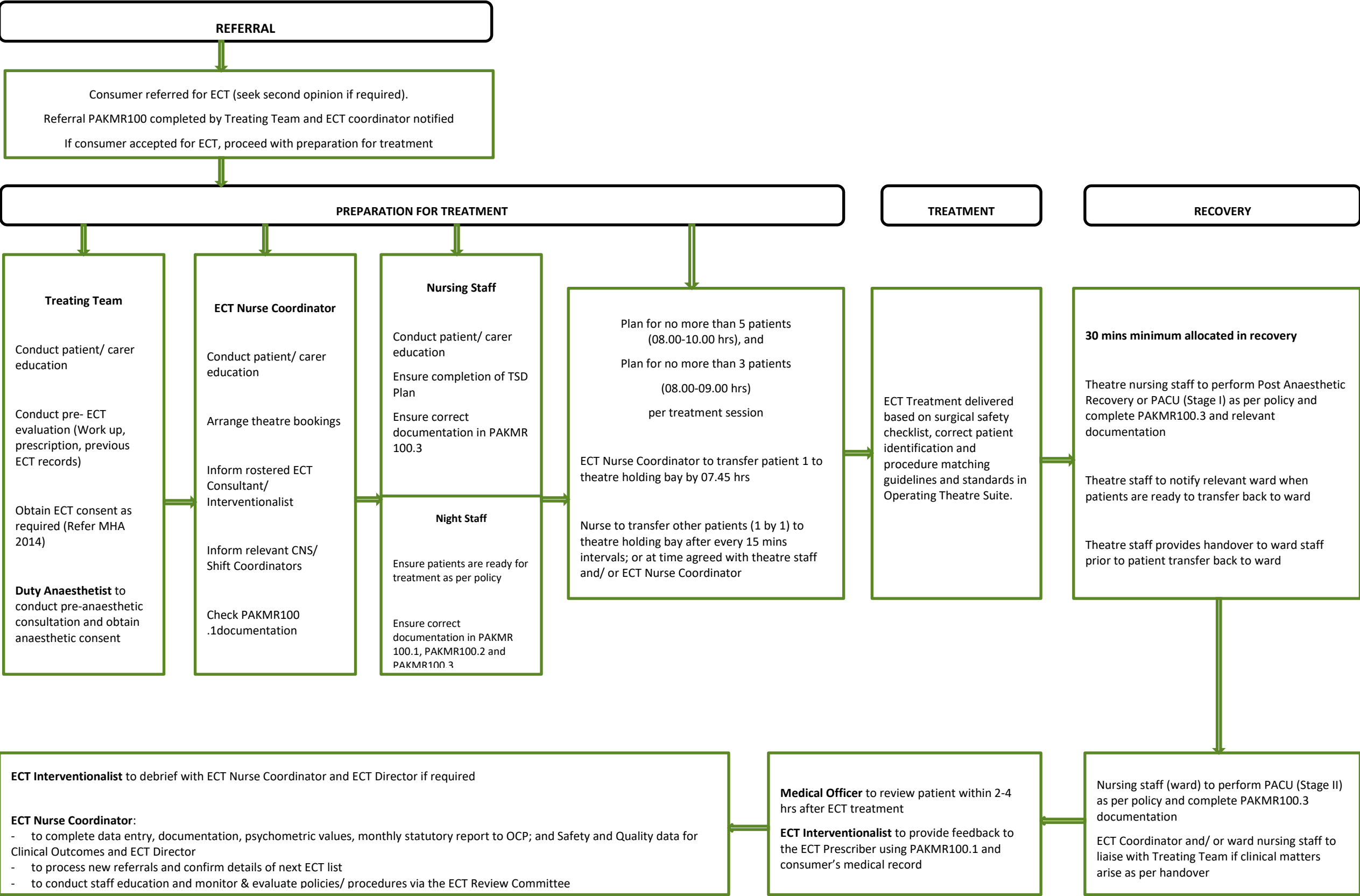
Version	Effective from	Effective to	Amendment(s)
V3.1	06 02 2026	19 03 2028	Minor update to include additional guidelines to refer to re anaesthetics

10. Authorisation

This policy has been authorised and issued by:

Authorised by	Dene Olive – A/Service Director Mental Health
Authorisation date	06 02 2026
Published date	06 02 2026

Appendix 1: ECT treatment flow chart (inpatients)



Appendix 2: ECT treatment flow chart (day patients)

